

MERCY VALE REGIONAL MEDICAL CENTER		Confidential · Printed 05/18/2026 1450	
KORVIN MERROW			MRN KM-6427819
DOB	02/18/1964 (62 yo) · Male	Code Status	Full Code
Allergies	Lisinopril (cough)	Admit / HD	05/18/2026 · HD1
Attending / Service	Elian Vossmere, MD · Hospitalist	Location / CSN	5 West Medicine · CSN-204418827

History & Physical

Author: Elian Vossmere, MD | Service: Hospitalist | Date of Service: 05/18/2026 1432 | Status: Signed

REASON FOR ADMISSION

Suspected urinary-source infection / sepsis physiology with altered baseline mental status, progressive weakness, poor oral intake, near-fall/lightheadedness, and AKI-on-CKD risk in a medically complex patient with HFrEF/CAD, CKD stage 3, type 2 diabetes, PMR with unclear prednisone taper history, and polypharmacy.

HISTORY OF PRESENT ILLNESS

Korvin Merrow is a 62-year-old man admitted from the ED after family brought him in for progressive weakness, poor oral intake, lightheadedness, near-fall, and confusion compared with baseline.

Family describes about 3 weeks of decline: less activity, reduced stamina, reduced food and fluid intake, increasing dependence, possible urinary symptoms, medication-management mistakes, and slower thinking. Today he nearly fell at home while standing/walking. Family prevented a fall and reports no head strike or loss of consciousness.

Patient reports fatigue and lightheadedness. He is able to answer questions but is slow and not fully reliable on medication details. He gives inconsistent history regarding prednisone use for PMR. He says he has been “tapering” but cannot clearly state the current dose or whether he has taken it consistently.

BASELINE FUNCTIONAL AND COGNITIVE STATUS

Per family, Korvin lives with family and is usually independent with basic activities, though slower due to chronic illness. He occasionally uses a cane for longer distances or bad days. Baseline cognition is mild age-related forgetfulness only; he is usually oriented and able to participate in routines. Family provides oversight for a complex medication regimen. Current weakness, medication mistakes, and cognitive slowing are a meaningful deviation from baseline.

PAST MEDICAL HISTORY

- HFrEF
- Coronary artery disease with remote PCI/stent
- Hypertension
- Hyperlipidemia
- CKD stage 3
- Type 2 diabetes mellitus
- Diabetic peripheral neuropathy
- Obstructive sleep apnea
- Polymyalgia rheumatica with chronic prednisone exposure and recent taper uncertainty
- Anemia of CKD
- Osteoporosis/osteopenia from chronic steroid exposure
- Class I obesity
- Chronic GERD / acid-suppression indication
- Chronic constipation tendency

SURGICAL / PROCEDURAL HISTORY

Remote PCI with coronary stent placement. Remote diagnostic sleep study confirming OSA. No CABG, pacemaker/ICD/CRT, dialysis access, major orthopedic repair, amputation, or temporal artery biopsy reported.

HOME MEDICATIONS (REPORTED — PENDING PHARMACY VERIFICATION)

Medication history is incomplete at admission and requires pharmacy verification. Reported from medication bag and family report. Doses not reliably confirmed at admission interview; prednisone dose/taper status uncertain.

Medication	Dose	Route	Frequency	Source / Status
Sacubitril/valsartan	Unverified	PO	Unverified	Family/med bag — pending pharmacy

Carvedilol	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Furosemide	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Spironolactone	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Empagliflozin	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Aspirin	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Atorvastatin	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Nitroglycerin (rescue)	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Metformin ER	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Insulin glargine	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Prednisone	<i>Uncertain</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Gabapentin	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Acetaminophen	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Pantoprazole	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Ferrous sulfate	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Alendronate	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Calcium / vitamin D	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Cholecalciferol	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Polyethylene glycol	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy
Senna	<i>Unverified</i>	PO	<i>Unverified</i>	Family/med bag — pending pharmacy

ALLERGIES

Lisinopril — cough (ACE-inhibitor intolerance).

PHYSICAL EXAM

Admission vital signs recorded in the nursing flowsheet (see Flowsheet, FI-W12). Bedside examination documented separately; no acute focal deficit reported by ED at handoff. Findings to be reassessed on inpatient rounds.

ASSESSMENT AND PLAN

1. Suspected urinary-source infection / sepsis physiology. Initial ED frame is clinically reasonable.
2. Altered baseline mental status, likely multifactorial: infection/acute illness, poor intake/volume status, medication burden, renal/metabolic risk, sleep/illness effects, possible steroid-related physiology.
3. Progressive weakness and near-fall/lightheadedness, multifactorial.
4. Poor oral intake with dehydration and AKI-on-CKD risk.
5. HFrEF/CAD with high-risk medication reconciliation needs during acute illness.
6. Type 2 diabetes with poor intake and renal-risk considerations.
7. PMR with unclear prednisone taper / source-of-truth problem.
8. Functional decline and discharge-safety concern already present on admission.

Plan:

- Admit to inpatient medicine; continue infection-oriented management and monitor response.
- Cautious volume strategy given HFrEF and CKD.
- Pharmacy medication reconciliation and refill-history review.
- Hold or reassess renal/hemodynamic-sensitive chronic medications until renal function, intake, and hemodynamics are clearer.
- Clarify prednisone history via med reconciliation, pharmacy history, family input, and outpatient rheumatology records.
- Monitor mental status, intake, mobility; PT/OT evaluation once stabilized.
- Keep family involved for baseline function and medication-management history.

ADMISSION REASONING

The suspected infection diagnosis explains part of the admission but does not yet explain the entire decline. The admission plan avoids premature closure and preserves medication, steroid-source, renal, cardiac, functional, and family-safety questions for reassessment as the patient evolves.

Electronically signed by Elian Vossmere, MD (Hospitalist Service) on 05/18/2026 1432